

1. Executive Summary

Dr Quick is an on-demand, pay-per-consult, no-membership GP video consultation marketplace that connects patients with doctors in real-time. Patients open the app, request a GP, and are matched live to an available doctor — like Uber — rather than booking a fixed future slot as with every existing UK private GP service.

The venture sits at the intersection of two documented, opposing failures in English healthcare: patients cannot get timely GP access, while a large and growing share of qualified GPs cannot find enough NHS work. A BMA survey of 1,000+ GPs found that 15% can find no GP work at all and 56% want to work more NHS hours but cannot find the opportunity. Dr Quick's core proposition is to connect this underused supply directly to the patients the NHS and existing private GP apps are not reaching fast enough.

The UK private-pay GP market is valued at approximately £1.6bn, with private consultations rising from 3% to 13% of all GP consultations over the past two decades. Dr Quick will be the first to combine (a) a genuinely live, on-demand matching experience and (b) a supply-sourcing strategy built explicitly around underemployed and locum GPs, rather than premium-rate gig work layered on top of already-busy doctors.

The model forecasts revenue growing from £2.46m in Year 1 to £21.6m by Year 3, reaching EBITDA breakeven in Year 2 and a 17.6% EBITDA margin by Year 3.

2. The Problem

2.1 Patient access is failing

Private GP consultations have more than quadrupled over 20 years as patients pay to bypass NHS waiting times.

2.2 Qualified GPs cannot find work — a paradox the government has publicly conceded

The Department of Health and Social Care has itself described inheriting "a ludicrous situation where patients can't get a GP, yet qualified GPs couldn't get a job."

- 15% of GPs can find no GP work at all; a further 56% want more NHS hours but cannot find them.
- 60% of GPs report falling pay rates for GP work over the past year; 69% report stress or anxiety tied to under- or unemployment.
- FTE GP job adverts in England fell 45% between Q1 2022/23 and Q2 2025/26 (919 to 504), even as newly-qualified GPs rose 49% (2015–2024).

This is the paradox Dr Quick is built to close: real, documented, government-acknowledged spare capacity on the supply side, and real, documented, worsening access failure on the demand side.

3. The Solution

Dr Quick is a two-sided marketplace:

- Patient side: request a GP video consultation on demand, matched live, no membership or subscription required, pay per consult.
- Supply side: GPs — sourced primarily from the underemployed/locum pool documented above — join as self-employed contractors, set availability flexibly, and are paid per consultation with no minimum commitment, directly mirroring the DocTap contractor model (£24–£33 per 15-minute slot) but pitched at a workforce with materially worse existing alternatives.

3.1 The dynamic-pricing mechanic

Dr Quick's differentiator is a live, Uber-style matching experience: patients see a real-time queue/ETA rather than a static appointment grid, and pricing flexes with real-time supply and demand rather than sitting on a fixed rate card as with every incumbent (DocTap, tapGP, Bupa PAYG, London Doctors Clinic, GPDQ).

4. Market Opportunity

The UK private GP market is worth £1.6bn 2026, with private consultations having risen from 3% to 13% of the total over the past two decades, driven substantially by NHS waiting times

Dr Quick's addressable slice is the pay-per-consult, no-membership tier of that £1.6bn — the same tier already occupied by DocTap, tapGP, London Doctors Clinic and Bupa's self-pay option. This plan's Year 3 revenue of £21.6m represents roughly 1.3% of the total private GP market — a deliberately conservative share for a still-young, national operator.

5. Competitive Landscape

This is a mature category. The table below sets out the closest comparables and how Dr Quick differs.

Competitor	What they do well	Where they fail
DocTap	Pay-per-consult, no membership; GPs paid per appointment as self-employed contractors	Fixed price, fixed appointment slots — no live matching or dynamic pricing
tapGP / London Doctors Clinic	Pay-per-consult, app/video based	Fixed price, fixed slots; narrower service scope
Push Doctor (now Square Health)	First UK video-GP platform at scale, 5.4m patients reached	Lost ~£8m in FY to July 2021 and was sold; 2017 CQC failure on age/ID checks — cautionary precedent for scaling consumer video-GP
GPDQ	Originated the literal "Uber for GPs" positioning in 2016	Drifted to NHS/ICB/corporate B2B contracts by 2025–26; consumer on-demand model did not scale on its own
HealthHero / Livi / insurer digital GP	Massive scale (HealthHero: 35m lives) via B2B2C insurer and employer bundling	Not a consumer-choice product; patient has no direct relationship with the brand
Lantum / My Locum Manager	Already digitally matches underemployed/locum GPs to shifts at scale (Lantum: 17,500+ GPs, 400–500 shifts/month in one ICB alone)	Solves the practice's staffing problem (B2B), not the patient's access problem (B2C) — adjacent, not competing, but could add a consumer layer faster than we could build their compliance infrastructure

6. Go-to-Market & Rollout Plan

Supply is recruited directly from the documented underemployed-GP pool (BMA/Nuffield data, Section 2) via targeted outreach to GPs on ARRS fixed-term contracts, recent CCT graduates who did not secure a practice post, and locums already registered on platforms such as Lantum who want additional, flexible income.

7. Financial Plan

Full month-by-month Year 1 build, 3-year annual P&L, and unit economics are provided in the accompanying workbook (Dr_Quick_Financial_Model.xlsx). Headline figures:

Metric	Year 1	Year 2	Year 3
Total consults	56,850	193,450	428,875
Blended avg. price / consult	£43	£48	£50
Revenue	£2.46m	£9.21m	£21.64m
Gross profit (43% margin)	£1.06m	£3.96m	£9.31m
Total operating expenses	£1.33m	£3.05m	£5.51m
EBITDA	(£0.27m)	£0.91m	£3.80m
EBITDA margin	(10.9%)	9.8%	17.6%

- Base off-peak consult price £32, blended average revenue per consult £43 in Year 1 rising to £50 by Year 3 as the pricing algorithm matures.
- GP payout fixed at 55% of consult revenue — in line with the DocTap benchmark (£24–£33 per 15-minute slot) — plus a supply-side surge kicker to protect fill-rate at peak without passing the full surge to patients.
- Gross margin held constant at 43% across all three years; margin improvement over the period comes entirely from operating leverage (headcount and marketing efficiency), not from cutting GP pay.
- Marketing spend at 22% of revenue in Year 1 (heavy launch acquisition), tapering to 12% by Year 3 as repeat usage and word-of-mouth mature.
- Cumulative 3-year EBITDA of approximately £4.4m; Year 1's negative EBITDA of (£0.27m) includes a one-off £45,000 CQC registration and legal set-up cost.

Unit economics (Year 1, blended): contribution margin per consult of roughly £18 (43% of £43), a CAC of £16, and a payback period on acquisition spend of well under one year assuming 1.8 repeat consults per patient/year.

8. Regulatory Path

Any platform providing regulated medical consultations to the public must itself be CQC-registered

9. Key Risks & Mitigations

Risk	Mitigation
Consumer willingness-to-pay ceiling — UK patients default to free NHS care in the vast majority of cases (Push Doctor's losses despite 5.4m patients reached)	Keep marketing spend efficient and targeted at moments of acute NHS failure (111 queue, same-day appointment) rather than broad brand marketing; B2B if CAC underperforms
ARRS or a future NHS workforce policy absorbs the underemployed-GP pool before Dr Quick scales	Build direct relationships with GPs early (referral incentives, faster onboarding, better take-home than ARRS) rather than relying on the pool being permanently uncontested
Lantum, DocTap or another incumbent adds live-matching or a similar underemployed-GP narrative	Narrative and workforce-sourcing are not a structural moat; the real strength is from execution speed and brand trust
CQC inspection failure on safety/verification grounds (Push Doctor precedent)	Build age/ID verification and clinical escalation pathways to CQC standard from day one, not as a post-launch fix

10. Recommended Next Steps

- Confirm with Dr Sick's Responsible Officer whether Dr Quick can launch under the existing CQC registration, and what variation (if any) is required.
- Stress-test the repeat-consult-rate and CAC assumptions with a small, unadvertised London pilot before committing to the full Year 1 marketing spend.
- Legal and clinical governance review of the surge-pricing mechanic specifically — cap levels, disclosure requirements, and a pre-drafted response to foreseeable press criticism — before it goes live, not after.
- Direct outreach to 20–30 underemployed/ARRS GPs to validate the supply-side pitch and payout rate before building the full GP-facing app.